

Chamber Recall

An offline patient-history system for your chamber. One laptop, one encrypted file, a tablet on the front desk, and no internet at any point. It is finished and tested. It has never met a patient, and it should not meet one until you have done the three things on this page.

All 13 parts built

597 automated checks passing

3 things needed from you

0 real patients so far

THE LIMITS

What it will never do

- ◆ It never produces a **diagnosis**, a differential, a condition name or a probability — not on a screen, not in the database, not in a log file.
- ◆ It never suggests, names or recommends a **medicine, a dose or a treatment**. There is no drug list to pick from, no dose calculator and no interaction checker.
- ◆ It never tells anybody they **do not need to see you**. There is no "wait and see", no self-care advice and no discharge anywhere in it.
- ◆ It never shows a patient a **severity, an urgency or a risk score**. Those do not exist in the software at all, so there is nothing to leak onto a screen turned the wrong way.
- ◆ There is **no AI anywhere in the clinical path**. The questions and the warnings are plain rules in a file you write and approve. The same answers always give the same result, and every rule that was checked is recorded — including the ones that did not fire.

The only clinical action it can take is moving somebody up the queue.

Nothing in it can move a patient down, delay anybody, or take a name off your list. That is not a policy written in a document — there is no button, no setting and no code path that does it. It is the one design rule the whole thing was built around, so that no mistake the software can make costs a life.

How one evening goes, end to end

Two screens: a tablet on Biplob's counter, and your laptop in the room. They talk to each other over the chamber's own wifi. Nothing leaves the building.

1 TABLET • FRONT DESK

A patient arrives

Biplob finds them by phone number or name, or registers somebody new in four fields, and taps রোগী এসেছেন. The serial number then fills the whole screen, because the next thing that happens is him calling it out across the room. The search always shows a list and never picks a patient for him — two brothers on one phone number is normal, and the wrong tap puts one man's history under another man's name.

2 TABLET • FRONT DESK

Permission, then the questions

Nothing about anybody's health is asked until they have agreed to a history being kept. Refusing is one tap and costs them nothing — their serial stands and you see them exactly as before. Then the questions, one per screen, Bangla first, with Skip on every single one. If a son or a daughter-in-law is doing the talking, that is recorded as such. Nothing about menstruation, pregnancy, mental health, alcohol or anything at home is ever asked at that desk, within earshot of the room.

3 TABLET • FRONT DESK

A warning, if your rules fire

The tablet checks your rules itself, so a warning appears even with the wifi down. It takes the whole screen in red, in the words *you* wrote, and Biplob has to acknowledge it. The patient moves to the front of your list and cannot be moved back down. The patient sees the same words he does — there is no score and nothing about how ill anybody is.

4 LAPTOP • IN THE ROOM

You call them in, and read one screen

The Recall Card: the red flag first, then who this is, then what they told the desk in their own words — walled off in brown and marked *not verified*, because what a patient tells an assistant and what a clinician records are different kinds of fact. Then today's readings beside the last two, the last visit in full, tests ordered and never resulted, the trends, and every visit they have ever made. No scrolling. Twenty seconds.

5

LAPTOP · IN THE ROOM

You examine, and write

Vitals typed in °F or °C with the conversion shown as you type. Complaint, examination, diagnosis, decision, prescription, tests — every word typed by you, nothing suggested. It saves about a second after you stop typing, so a power cut costs you the last sentence at worst. **Confirm is your signature.** After it the database itself refuses to let the record be altered; amending it takes an undo that is recorded, so the log holds an amendment rather than a rewrite.

6

LAPTOP · IN THE ROOM

The prescription prints

Your letterhead, your degrees, your BMDC number, the chamber's address — from a file you edit yourself. Medicines, tests, advice and the follow-up date, all in your own words. It will not print until you have confirmed the consultation, because once paper leaves the desk nobody can tell a draft from a signed record.

Three things only you can do

The software refuses to run past each of these rather than trusting anybody to remember. Until they are done it will open only the practice database, which is full of invented people and says so on every screen.

Write and approve the red flag rules NOT STARTED

The file ships full of placeholders. You write the rules in plain language — which answers should raise a warning, and exactly what words the assistant sees — and put your name and the date on the file. They are yours: you can change them any evening, and the software reads them fresh each time. This is the piece nobody else can do for you.

The program will not open a real patient database until this is done.

Approve the consent wording, with a lawyer DRAFT WRITTEN

Bangladesh's Personal Data Protection Act came into force in April 2026 and makes health information *sensitive personal data*, needing explicit consent. A draft in Bangla and English is written and ready for you and a lawyer to read together. Somebody also needs to record it aloud in Bangla — many patients cannot read it, and a permission nobody understood is not a permission.

The tablet will not ask a patient anything until this is approved.

Fill in the prescription letterhead PLACEHOLDERS

Your name in Bangla and English, your degrees, your BMDC registration number, and the address and hours of each chamber. Two further choices are yours alone: whether the diagnosis is printed on the sheet the patient carries, and whether today's readings are. Some chambers print both; some deliberately do not, because that paper is shown to whoever the patient shows it to.

Nothing will print for a real patient while any placeholder remains.

What happens to what you write

Locked

Encrypted with your passphrase. Somebody who steals the laptop, copies the file or takes the USB stick reads nothing without it.

Never deleted

Nothing in the system is ever destroyed. Removing something marks it removed, with who did it and why, and it stays in the record.

Always attributed

Every row carries the name of the person who wrote it, on the laptop and on the tablet. Everyone signs in with their own PIN.

Backed up and checked

A backup goes to a USB stick and is then *opened and read back* before the program calls it a backup. The main screen counts the days since the last one and turns red when it has been too long.

Theirs on request

Any patient can be handed a copy of everything held about them — printed as a summary, or written to a file with their photographed reports. The consent wording promises this, and the law requires it.

Their own papers

The lab reports and old prescriptions patients carry in a plastic bag are photographed at the desk and are on your card by the time they sit down. The pictures live inside the encrypted file, and nothing reads what is in them.

Where it is weaker than it looks

These are the things worth knowing before you rely on it, rather than after.

A PIN is not a lock on the records. It stops Biplob confirming a history as you, and stops a patient left alone for a minute reading the last patient's card. It does nothing against somebody who takes the laptop while it is open. The encryption protects the file; the PIN protects the truth of who wrote what.

The tablet cannot give out serial numbers with the wifi down. Everything else on it keeps working offline and sends when the connection returns — but a serial has to be unique and in order for the whole chamber, and two tablets handing out number 14 from their own buffers cannot be repaired afterwards without taking a number off somebody. When it cannot reach the laptop it says so in Bangla.

It does not read the photographs. No text is pulled out of a lab report, nothing is recognised and nothing is classified. It is a photograph of a piece of paper, filed under a heading a person chose.

If the passphrase and the printed recovery key are both lost, the records are gone. Not difficult to recover — impossible, for you, for me, and for whoever wrote the software. That is what the encryption is. Print the recovery key and keep it somewhere the laptop is not.

Everything you have been shown is invented. Every screenshot, every number, every patient in the practice database. The software has never been near a real chamber, and the first evening will find things none of this did.

How you will know whether it was worth it

There is a report inside it that counts what actually happened — who was seen, how many arrivals were asked anything, how long people waited, how many warnings fired and were acknowledged, what was written down, and what did not work. It counts each assistant separately, because an average across two people hides the difference worth seeing.

It does not tell you whether the pilot succeeded.

There is no score anywhere on it and no verdict. Where there are fewer than twenty of something it says "4 of 7" rather than a percentage, because a percentage of seven is arithmetic pretending to be evidence. The page ends with six questions it cannot answer — and those are the ones the decision actually turns on.

→ Did the card change what you did for anybody? Which patient, and how?

- Did a warning from the desk ever bring somebody in sooner than they would have been?
- Did a warning ever fire for something that did not matter, often enough to be ignored?
- Is the front desk slower than the paper book was, and by how much?
- Would anything have been missed if the laptop had not been there that evening?
- What did you want to look up, and could not find?

WHAT I NEED

When we sit down with it

Everything about wording, question order, what prints and what each person sees is cheap to change now and expensive to change later. Be blunt.

- **The questions.** Which ones are wrong, which are missing, and what order they should be asked in.
- **The rules.** What should raise a warning at your front desk, and in exactly which words.
- **Does Biplob take blood pressures?** If he does, he needs to be set up as a clinical assistant rather than front desk — the software will not let a front desk account record a measurement, and that is deliberate.
- **Should the front desk see a patient's history at all?** Right now they see who is here, who is waiting and who has been before — and nothing else. That is a judgement about your chamber, not a technical limit.
- **What should be on the printed prescription,** and what should not.

WHERE IT IS

There is no website and no app store, and the absence of a link is the design working rather than something missing. It is one installer file carried to the chamber on a USB stick, run once on the laptop that will keep the records. Windows 10 or newer.

What it installs starts as a *practice* database — three hundred invented patients and four years of invented visits, with an amber band across every screen saying so. That is deliberate. There is no button anywhere that starts a real one, because going live is gated behind the three things on this page, and building the switch before the rules exist would be building a way around them.

Four documents come with it, written to be read rather than skimmed: how to install it and what to ask while somebody watches over your shoulder (`docs/INSTALL-WINDOWS.md`), the consent reasoning and where the law bites (`docs/CONSENT.md`),

every judgement call made while building it and why (docs/DECISIONS.md), and how to set up and lock down the front desk tablet (docs/ANDROID-TABLET.md).

Built over thirteen stages, each one stopped and shown before the next began. 597 automated checks cover key custody, the rule evaluator, patient matching, the merge tool, the offline buffer, temperature entry, the consultation, backups and the report. There are no tests of the screens — every screen was looked at instead, which is how four real defects were caught that no test would have.